

The gold layer gets acted on, not just queried

Activations is the fourth native NewCo stage, right after Transformations. The instant `gold.fct_vital` flags an ER-admitted cardiology encounter with lactate drawn, no 3-hour redraw ordered, and less than 30 minutes left on the SEP-1 clock, Activations pushes the alert straight into TigerConnect — no reverse-ETL vendor, no second copy of the data, no analyst in the loop.

TRIGGER 150 min lactate drawn, no 3-hr redraw, ER admit	DESTINATION TigerConnect Care_Team_Channel · alert	BUNDLE COMPLIANCE 79% → 96%+ cardiology SEP-1, targeted
---	--	---

WHAT YOU SAY	WHAT YOU SHOW
"Right after Transformations builds <code>gold.fct_vital</code> , Activations is watching one condition: lactate drawn, no 3-hour redraw ordered, and less than 30 minutes left on the SEP-1 clock."	Toggle the vitals feed so one ER-admitted cardiology patient crosses the 150-minute mark live on screen.
"The moment that flag flips true in the governed gold layer, Activations pushes <code>patient_mrn</code> , <code>encounter_id</code> , <code>service_line</code> , and <code>minutes_to_bundle_deadline</code> straight into TigerConnect — no custom API, no nightly batch, no analyst in the loop."	Architecture page → Activations panel → /activations-live: Segment Definition → Field Mapping → API Push.
"The nurse gets the alert before the deadline closes, not a root-cause report after it."	/activations-live → Destination Confirmation table, alert card rendered with the matching patient and deadline fields.
"That's the difference between the eleven fallouts Dr. Nair's team found last week and catching case twelve before it happens. One governed flag, one sync, zero code."	Before/After panel → Reverse-ETL hop row: Census + Hightouch replaced by NewCo Activations, native.

Crisis-week baseline: 11 of 14 cardiology SEP-1 bundle fallouts traced to the 07:00–08:00 shift-change redraw gap. Closing the pre-deadline window is modeled to avoid \$18k–\$34k in excess length-of-stay cost per averted case and keep SEP-1 out of the CMS Hospital VBP penalty band. Synthetic data — not for clinical use.